



VIJAYA DIAGNOSTIC CENTRE®

Plot No.2A, Street No. 1, Kakateeya Nagar, Habsiguda, Hyderabad, Telangana 500007

TEST REPORT

Name : Mr. SOMAYAJULU

Age/Gender : 51 Years / Male

Registration ID : 250880001675

Ref. By : Self

Sample Type : Serum

Registered on : 25-Jan-2025 10:37

Collected on : 25-Jan-2025 10:55

Released on : 25-Jan-2025 21:07

Printed on : 25-Jan-2025 23:08

Regn Centre : Yaprak - 88

LIVER FUNCTION TEST (LFT-A)

TEST NAME	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Total Bilirubin <i>Method: Dichlorophenyl Diazonium Tetrafluoroborate</i>	: 11.0	mg/dL	0.3 - 1.2
Direct Bilirubin <i>Method: Dichlorophenyl Diazonium Tetrafluoroborate</i>	: 3.9	mg/dL	< 0.2
Indirect Bilirubin <i>Method: Calculation</i>	: 7.1	mg/dL	0.3 - 1.0
SGPT/ALT <i>Method: IFCC without P-5-P</i>	: 25	U/L	0 - 50
SGOT/AST <i>Method: IFCC without P-5-P</i>	: 60	U/L	0 - 50
Alkaline Phosphatase <i>Method: Kinetic PNPP-AMP</i>	: 223	U/L	43 - 115
Total Protein <i>Method: Biuret</i>	: 5.9	gm/dL	6.6 - 8.3
Albumin <i>Method: Bromocresol Green (BCG)</i>	: 2.8	gm/dL	3.5 - 5.2
Globulin <i>Method: Calculation</i>	: 3.1	gm/dL	1.8 - 3.6
Protein A/G Ratio <i>Method: Calculation</i>	: 0.9		0.8 - 2.0
Gamma Glutamyl Transferase <i>Method: UV Kinetic</i>	: 21	U/L	0 - 55

Interpretation / Comments :

- Liver function test aids in the diagnosis of various pre hepatic, hepatic and post hepatic causes of dysfunction like hemolytic anemias, viral and alcoholic hepatitis and cholestasis of obstructive causes.
- The test encompasses hepatic excretory, synthetic function and also hepatic parenchymal cell damage.
- LFT helps in evaluating severity, monitoring therapy and assessing prognosis of liver disease and dysfunction.



DR. SYED SHABBAR MASIH

MD BIOCHEMISTRY

Registration No: TSMC/FMR/44589



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TEST REPORT

Name : **Mr. SOMAYAJULU**
Age/Gender : **51 Years / Male**
Registration ID : **250880001675**
Ref. By : **Self**
Sample Type : **Whole Blood - EDTA**

BirthDate : **19-Aug-1973**

Registered on : **25-Jan-2025 10:37**
Collected on : **25-Jan-2025 10:55**
Released on : **25-Jan-2025 18:22**
Printed on : **25-Jan-2025 23:08**
Regn Centre : **Yapral - 88**

COMPLETE BLOOD PICTURE (CBP)

<u>TEST NAME</u>	<u>RESULT</u>	<u>UNIT</u>	<u>BIOLOGICAL REFERENCE INTERVAL</u>
Haemoglobin <i>Method: Photometric Measurement</i>	: 10.4	gm/dL	13.0 - 17.0
Total RBC Count <i>Method: Coulter Principle</i>	: 3.1	millions/cumm	4.5 - 5.5
Packed Cell Volume / Hematocrit <i>Method: Calculated</i>	: 30.1	%	40.0 - 50.0
MCV <i>Method: Derived From RBC Histogram</i>	: 98.0	fL	83.0 - 101.0
MCH <i>Method: Calculated</i>	: 33.8	pg	27.0 - 32.0
MCHC <i>Method: Calculated</i>	: 34.5	gm/dL	31.5 - 34.5
RDW <i>Method: Derived From RBC Histogram</i>	: 20.6	%	11.6 - 14.0
Total Leucocytes (WBC) Count <i>Method: Coulter Principle</i>	: 2900	Cells/cumm	4000 - 10000
<u>Differential count</u>			
Neutrophils <i>Method: VCSn Technology and Microscopy</i>	: 61	%	40 - 80
Lymphocytes <i>Method: VCSn Technology and Microscopy</i>	: 26	%	20 - 40
Eosinophils <i>Method: VCSn Technology and Microscopy</i>	: 4	%	1 - 6
Monocytes <i>Method: VCSn Technology and Microscopy</i>	: 8	%	2 - 10
Basophils <i>Method: Optical / Resistive DC & Microscopy</i>	: 1	%	0-2
<u>Absolute Leucocyte Count</u>			
Absolute Neutrophil Count <i>Method: Calculated</i>	: 1769	Cells/cumm	2000 - 7000
Absolute Lymphocyte Count <i>Method: Calculated</i>	: 754	Cells/cumm	1000 - 3000
Absolute Eosinophil Count <i>Method: Calculated</i>	: 116	Cells/cumm	20 - 500
Absolute Monocyte Count <i>Method: Calculated</i>	: 232	Cells/cumm	200 - 1000

* Suggested Clinical Correlation, If Necessary Kindly Discuss with signatory



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COMPLETE BLOOD PICTURE (CBP)

TEST NAME	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Platelet Count <i>Method: Coulter Principle and Microscopy</i>	: 60000	Cells/cumm	150000 - 410000
Peripheral Smear RBC <i>Method: Microscopy of Leishman stained smear</i>	: Normocytic Normochromic with moderate anisocytosis		
WBC <i>Method: Microscopy of Leishman stained smear</i>	: Moderate Leucopenia		
Platelets <i>Method: Microscopy of Leishman stained smear</i>	: Moderate thrombocytopenia		
Remarks	: Suggested correlation with iron studies including serum ferritin, vitamin B12 and folate assays		



K. Dantla

DR. KAPAGANTI VAMSI KRISHNA
MD PATHOLOGY
Registration No: TSMC/FMR/30732



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Ref. By : Self

Sample Type : Serum

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URIC ACID

TEST NAME	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
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Uric Acid

: 2.0

mg/dL

3.5 - 7.2

Method: Uricase-Peroxidase

Interpretation / Comments :

- Useful for monitoring therapeutic management of gout and chemotherapeutic treatment of neoplasms.



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UREA

TEST NAME	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Urea	11	mg/dL	17 - 43

Method: Urease/GLDH

Interpretation / Comments :

- In conjunction with serum creatinine, urea level aids in differential diagnosis of pre-renal, renal and post-renal hyperuremia.



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CREATININE

TEST NAME	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Creatinine	: 0.6	mg/dL	0.7 - 1.2

Method: Jaffe Kinetic IDMS traceable

e-GFR (Glomerular Filtration Rate)

: 121.3

ml/min/1.73 m²

Method: Calculation, CKD EPI equation

Normal kidney function : ≥ 90
(Please refer to Interpretations for reference ranges of e-GFR in different stages of CKD and also average e-GFR based on age.)

Interpretation / Comments:

Stages of chronic kidney disease (CKD)		
Stages	Description	e-GFR
1	Possible kidney damage (eg: Proteinuria) with normal kidney function	≥ 90
2	Mild loss of kidney function	60 - 89
3a	Mild to moderate loss of kidney function	45 - 59
3b	Moderate to severe loss of kidney function	30 - 44
4	Severe loss of kidney function	15 - 29
5	Kidney failure	< 15

Chart for average e-GFR based on age	
Age (Yrs)	Average e-GFR
20 - 29	116
30 - 39	107
40 - 49	99
50 - 59	93
60 - 69	85
≥ 70	75



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CREATININE

- Serum Creatinine is useful in the diagnosis of renal insufficiency and is more specific and sensitive indicator of renal disease than serum Urea /BUN.
- Use of simultaneous Urea / BUN and creatinine levels provide more information in the diagnosis of renal insufficiency.
- GFR is generally considered the best index of overall kidney function.
- Chronic kidney disease is defined as the presence of persistent and usually progressive reduction in GFR.
- Repeated determination of GFR in conjunction with creatinine assay establish whether the patient has stable or progressive disease.
- GFR is useful for people with chronic kidney disease (CKD) and those with risk factors for CKD (diabetes, hypertension, cardiovascular disease and family history of kidney disease) to assess the kidney function.
- The CKD-EPI equation is the most widely used IDMS traceable equations for estimating GFR in patients above 18 years of age. This equation includes variables for age and gender, and it may be observed that Kidney may be involved despite a serum creatinine concentration appearing to be within or just above the Biological Reference Interval. The results of e-GFR by CKD-EPI equation are normalized to 1.73 m² body surface area. CKD-EPI equation is not valid for individuals under 18 years of age.
- Limitations of CKD-EPI equation includes imprecise estimates in some individuals especially those suffering from physiologic limitations of creatinine as filtration markers, thus showing large difference between measured GFR and e-GFR (estimated GFR).
- Estimates for GFR based on serum creatinine will be less accurate for patients at the extremes of muscle mass (such as frail elderly, critically ill, cancer patients) and also those with unusual diets, sudden acute renal failure, patients on dialysis and patients with severe liver disease.
- Confirmatory tests with exogenous measured GFR or directly measured creatinine clearance should be performed for such individuals.
- The influence of creatinine measurement imprecision at low creatinine concentrations (and hence high e-GFR) has a possible contribution to the variability at higher e-GFR values.



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ELECTROLYTES

TEST NAME	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Sodium Method: Indirect ISE	: 133	mmol/L	136 - 146
Potassium Method: Indirect ISE	: 3.9	mmol/L	3.5 - 5.1
Chlorides Method: Indirect ISE	: 104	mmol/L	101 - 109

Interpretation / Comments :

- Sodium : Levels of sodium when evaluated with electrolytes aid in assessing acid base balance, water balance and water intoxication.
- Potassium : Useful in evaluation of electrolyte balance, cardiac arrhythmia, muscular weakness, hepatic encephalopathy and renal failure.
- Chloride : Useful when assayed along with sodium, potassium and bicarbonate in assessment of electrolyte, acid base and water balance.



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DEPARTMENT OF RADIOLOGY AND IMAGING SCIENCES ULTRASOUND ABDOMEN

- History** : Chronic liver disease.
- Liver** : Normal in size (11.9 cm) and shows coarsened echotexture with mild surface nodularity. There is no evidence of intrahepatic biliary dilatation. CBD appears normal. Portal vein (PSV : 23 cm/sec) appear normal.
- Gall Bladder** : Partially distended. Mild wall thickening measuring ~ 6 mm noted.
- Pancreas** : Normal in size, shape and echopattern.
- Spleen** : Enlarged in size (16.6 cm) with normal echopattern.
- Kidneys** : Right kidney : 113 x 41 mm ; Left kidney : 105 x 39 mm
Both kidneys are normal in size, shape and echopattern.
Cortico medulary differentiation is maintained.
Collecting system appears normal. No evidence of calculi.
- Urinary Bladder** : Well distended. No wall thickening seen. No evidence of calculi/mass.
- Prostate** : Size : 26 x 38 x 28 mm (Volume : 15 cc)
Normal in size, shape and echotexture.
- Other Findings** : Evidence of mild anechoic free fluid noted involving perihepatic region and pelvis - S/o mild ascites.



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Opp.SBI Bank, Yapral Main Road, Bapuji Nagar, Yapral, Secunderabad, Telangana-500094

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DEPARTMENT OF RADIOLOGY AND IMAGING SCIENCES ULTRASOUND ABDOMEN

Impression

- : • **COARSENEDED ECHOTEXTURE OF LIVER WITH MILD SURFACE NODULARITY.**
• **GALL BLADDER WALL EDEMA.**
• **SPLENOMEGALY.**
• **MILD ASCITES.**
FEATURES SUGGESTIVE OF CHRONIC LIVER DISEASE.

- FOR CLINICAL CORRELATION AND ADVICE FURTHER EVALUATION IF INDICATED.



DR. VAISHNAVI VEMUGANTI
MBBS, MD
Registration No: 03838

----- End of Report -----